AR-09 Ventilator Emergencies

CLAIBORNE COUNTY EMS | MEDICAL-DIRECTOR APPROVED | SOURCE PDF | OCTOBER 17, 2026

Scope: Ventilator-dependent patient with alarm, failure, hypoxia, abnormal EtCO2, distress, or suspected airway/circuit problem. Treat the patient, not the alarm; never allow troubleshooting to delay manual ventilation.

Field flow

1. Baseline	Airway type, diagnosis, usual settings, usual SpO2/EtCO2, alarm meaning, and caregiver rescue plan.
2. Unstable?	If ventilation/oxygenation is inadequate or the cause is unclear: disconnect and BVM immediately.
3. Check	Oxygen, power/battery, circuit, connection/leak, kink, water, filter, settings, and alarm limits.
4. Resolve	DOPE and auto-PEEP; resume established settings only when safe or continue BVM and transport.

Provider-level actions

Level	Approved actions
EMT	Obtain caregiver baseline, check power/oxygen/circuit, assess tracheostomy/ETT, suction when indicated, use BVM if correction is not rapid, and bring ventilator to hospital.
AEMT	All EMT care plus advanced suctioning, tracheostomy/BIAD support, capnography, access when indicated, and troubleshooting assistance.
Paramedic	All prior care plus airway and physiologic management. Maintain functioning home settings when appropriate; avoid unneeded changes during transport.

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SAFETY, MEDICATION, AND LINKED-PATHWAY REFERENCE

Safety and escalation

• For asthma/COPD or prolonged exhalation, consider auto-PEEP: disconnect, manually ventilate slowly, allow full exhalation, then reassess.

• High-pressure alarm: kink, biting, secretions/plugging, bronchospasm, pneumothorax, or dyssynchrony. Low-pressure: disconnection, leak, cuff leak, loose tubing, or displaced airway.

• Suction when secretions/obstruction are suspected; preoxygenate when possible and never force a catheter.

• Transport with ventilator, power/battery, oxygen reserve, BVM, suction, spare tracheostomy equipment, and caregiver/information when feasible.

Linked pathways

AR-08 Post-Intubation / BIAD Management; AR-10 Tracheostomy Tube Emergencies; AR-11 Mechanical Ventilation, Adult.

Source control

• This controlled direct source PDF replaces the prior source file and matches approved Claiborne EMS app content released October 17, 2026.

• Use current Claiborne standing orders, formulary, equipment authorization, and Medical Control direction whenever they govern a case-specific decision.